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NAME: TEST PATIENT DELTA **AGE/SEX: 52 / F**
REFD. BY: TEST REFERRING DOCTOR **DATE: 15/10/2026**
UHID: TEST-004 **ACCESSION: ACC-MRI-LS-002**

MRI LUMBOSACRAL SPINE

CLINICAL HISTORY:

Chronic low back pain with left leg radiculopathy

TECHNIQUE:

Multiplanar, multisequence MRI of the lumbosacral spine was performed without intravenous contrast.
Sequences acquired: Sagittal T1, T2, STIR; Axial T2.

MEASUREMENTS:

Measurement	Value
L1-L2 Canal AP	14.5 mm
L2-L3 Canal AP	13.8 mm
L3-L4 Canal AP	13.2 mm
L4-L5 Canal AP	12 mm
L5-S1 Canal AP	11.5 mm

FINDINGS:

Alignment

Loss of normal lumbar lordosis.

No spondylolisthesis.

Vertebral Bodies

Normal heights.

Normal signal intensity.

Bone Marrow

Normal signal intensity.

Endplates

Normal.

Intervertebral Discs

Multilevel disc desiccation from L2-L3 through L5-S1.

L1-L2

Normal disc height.

No disc bulge or herniation.

L2-L3

Mild disc desiccation.

No disc bulge or herniation.

L3-L4

Moderate disc desiccation.

No disc bulge or herniation.

L4-L5

Moderate disc desiccation.

Diffuse disc bulge causing mild thecal sac indentation.

No significant neural foraminal narrowing.

L5-S1

Severe disc desiccation.

Posterior disc protrusion measuring approximately 4 mm in AP dimension.

Moderate left neural foraminal narrowing with contact of the left L5 nerve root.

Normal

Dr. Sugandha Priyadarshini
MD (Radiodiagnosis & Medical Imaging)

Radiological diagnosis is not always conclusive & often vary with clinical course of the disease or response to treatment. This report is not for medico-legal purpose.

Level	AP Diameter
L1-L2	14.5 mm
L2-L3	13.8 mm
L3-L4	13.2 mm
L4-L5	12.0 mm
L5-S1	11.5 mm

IMPRESSION:

1. Loss of lumbar lordosis.

2. Multilevel lumbar disc desiccation.

3. L4-L5 diffuse disc bulge with mild thecal sac indentation.

4. L5-S1 posterior disc protrusion with moderate left neural foraminal narrowing and left L5 nerve root contact.

5. Mild facet arthropathy at L4-L5 and L5-S1.

RECOMMENDATION:

Clinical correlation for left L5 radiculopathy. Consider targeted epidural steroid injection if conservative management fails.

REPORT

Radiological diagnosis is not always conclusive & often vary with clinical course of the disease or response to treatment. This report is not for medico-legal purpose.